Separating decision-rule failures from representation failures in external validation of diabetic retinopathy grading models

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Abstract

Diabetic retinopathy (DR) grading models lose performance on new populations, and the losses are usually attributed to the learned representation, although ordinal models convert scores into grades with a rarely examined decision rule. Under analysis plans committed in advance to version control, we evaluated ten development models on four external populations (APTOS 2019, DDR, DeepDRiD and SUSTech-SYSU), a model trained on APTOS in the reverse direction, two models trained on about 28 000 EyePACS images, five seeds and a published model's released predictions. We separated discrimination at each ordinal threshold from the mapping of scores to grades, and compared models under a matched decision rule that refits both models' cut-points identically. Ranking transferred everywhere (area under the curve for referable DR at least 0.914), but the score given the true grade moved in opposite directions: the same models over-graded APTOS and under-graded DDR, so a threshold fitted for 85% development sensitivity reached 99.33% and 63.38%. Refitting cut-points without retraining recovered the mild grade

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on APTOS and raised referral sensitivity in the reverse direction from 43.5% to 77.9%, but did not repair losses of ranking; label-free prior adaptation did not help. Contrary to two of our predictions, models differed more than deployable rules, and the mild-grade collapse was specific to one corpus. About 200 locally labelled images set a referral threshold with less than a one-in-five risk of missing an 80% sensitivity floor. External evaluations should report per-threshold discrimination and operating curves, and referral thresholds should be set locally before deployment.

Keywords: diabetic retinopathy, ordinal classification, decision threshold, external validation, dataset shift, recalibration, deep learning

1. Introduction

Diabetic retinopathy (DR) affects about one in five people with diabetes, an estimated 103 million adults in 2020 and a projected 161 million by 2045[1]. Deep learning systems that grade fundus photographs reached specialist-level agreement in development studies[2, 3], and autonomous screening devices have been authorised on the strength of prospective pivotal trials[4, 5]. The same class of system behaves very differently once it leaves the population it was built on: seven commercial algorithms applied to the same real-world images ranged from 51% to 86% in referable-disease sensitivity[6], and three algorithms evaluated against one reference standard in Indian public-health clinics ranged from 14% to 96% in specificity[7].

Such failures are usually read as statements about what the network has learned. A recent frozen-encoder benchmark found that two of three encoders, including the retinal foundation model RETFound, did not identify

a single mild case in an external corpus (grade-1 F1 of 0.000) and attributed the result to the encoders' representations[8]. An ordinal grading model does not output a grade, however. It outputs scores, and a *decision rule* — a set of cut-points — turns them into a grade and a referral. The common default, 0.5 on every cumulative output or an argmax, is a choice that is rarely reported or examined. If a model still ranks images correctly but its cut-points sit in the wrong place for a new population, it will look blind to a grade it in fact separates, and two models that differ mainly in where their defaults fall will look as if they differ in what they learned.

Parts of this idea are not new. Recalibration before deployment is standard advice for clinical prediction models[9, 10]. ORDER-DR, an ordinal DR model developed on APTOS, explicitly distinguished threshold-dependent grade assignment from threshold-independent risk ranking and noted that models trained on one public dataset can show threshold shift externally[11]. For binary referable DR across fundus cameras, a recent preprint showed that discrimination transfers while probability calibration degrades, mostly through prevalence, and that 100–200 labelled target images restore it [12]; concurrent work reports that calibration degrades unevenly across ordinal thresholds under domain shift[13]. What these studies do not provide is a decomposition across many models and several populations of how much of an external failure, and of an apparent difference between models, the ordinal decision rule accounts for; evidence on why the score-to-grade mapping moves; a rule for comparing models that removes it; and the label cost and risk of repairing it.

We provide these in a study whose analysis plans were committed to

version control before the corresponding analyses were run, reporting falsified predictions with the same weight as confirmed ones. Ten development models trained on 2,260 images were evaluated on four external populations — APTOS 2019 (India), DDR (China, 147 hospitals), DeepDRiD (Shanghai) and SUSTech-SYSU (Shenzhen) — and a model trained on APTOS was evaluated in the reverse direction. Two further models were trained on about 28 000 EyePACS images to test whether the findings depend on the size of the development set, five seeds of the reference model measured training noise, and the released predictions of ORDER-DR were reanalysed. The contributions are:

- a decomposition of external performance into discrimination at every ordinal threshold and the mapping from score to grade, which shows that the mapping moves in opposite directions on different populations through a shift in the score *given* the true grade;
- an account of what changing the decision rule can and cannot recover, including a negative result for label-free prior adaptation;
- the *matched decision rule*, a comparison rule under which both models' cut-points are refitted by the same procedure, with its record of changed, unchanged and reversed comparisons;
- the number of locally labelled images needed to set a referral threshold, with its probability of harm, and what the operating point means per 10 000 people screened.

2. Materials and methods

2.1. Study design and analysis plans

This is a retrospective modelling study on public, de-identified fundus photographs. An evaluation protocol (unit of independence, held-out sets, model selection on development data only, metrics and uncertainty) was fixed before the first training run. For the analyses reported here, the plan — hypothesis, control, analysis and the result that would falsify it — was committed to the public repository before the analysis was run: the derivation test of the macular-oedema grade (Supplementary Note 2), the nine journal outcomes O1–O9 (seed replicates, reverse direction, second external corpus, five-fold foundation model) and the revision outcomes A1–E2 (mechanism, label-free rules, sample size, two further external sets and the models trained on EyePACS). These are analysis plans prospectively committed to version control, not entries in a public registry; their commit identifiers are listed in Supplementary Table S1. The repository history begins on 31 August 2026, so the pre-specification of development runs trained before that date cannot be verified from it, and we do not rely on it. Reporting follows TRIPOD+AI[14] and CLAIM[15] (Supplementary Table S2).

2.2. Data

The *development pool* combined IDRiD[16, 17] (516 images) and Messidor-2[18, 19] (1,744 gradable images with adjudicated grades[20]), 2,260 images in total (Table 1; Fig. 1). The external sets, never used for training, model selection or threshold fitting, were APTOS 2019 (3,662 images, India, single grader), DDR[21] (12,522 gradable images, 147 hospitals in

China, seven graders, distributed at 512 pixels), the regular fundus images of DeepDRiD[22] (**??r2.deepdrid.n??** images of 500 patients, Shanghai) and SUSTech-SYSU[23] (**??r2.sustech.n??** images with an international-scale grade, Shenzhen). In the reverse direction APTOS was the training corpus and Messidor-2 and IDRiD were external. The EyePACS 2015 training set (35,126 images, single grader) was used to pretrain the development models and, at revision, as the development set of two further models. All corpora use the international 5-grade scale[24]; referable DR is grade $\geq$2 and, because macular oedema is not graded in the external sets, it is defined by DR grade alone. A content audit (256-bit difference hash after the training crop) found no development image within re-encoding distance of any external image (Supplementary Note 7). Patient identifiers are published only for DeepDRiD and EyePACS; Messidor-2 is distributed as pairs of images per examination, but the pairing file is not part of the mirrors used, so resampling elsewhere is by image (near-duplicates merged).

2.3. Models

All models share one design: a backbone, global average pooling and ordinal threshold heads that output four logits for $P(y > k)$[25], trained with masked binary cross-entropy, a grade-balanced sampler, AdamW, cosine decay, weight averaging and dihedral test-time augmentation (details in Supplementary Table S3). The reference model is a DenseNet121[26] pretrained on ImageNet and then EyePACS and fine-tuned at 448 pixels; the selected model replaces the backbone with EfficientNet-B3[27]. The other eight preregistered development models change one factor each (Table 2): resolution (224, 560 and native 640 pixels), a macula-centred crop, linear-probe-then-

fine-tune[28], CORAL and CORN heads[25, 29] and ConvNeXt-tiny[30]; the retinal foundation model RETFound[31] was fine-tuned separately. Development results are out of fold over a frozen 5-fold split stratified by corpus and grade; external predictions average the five fold models. The models trained on EyePACS (DenseNet121 and EfficientNet-B3, 448 pixels, 15 epochs from ImageNet) used a patient-grouped split, trained on four folds and used the fifth (**??eye.dn.devn??** images) for checkpoint selection and for fitting development rules.

2.4. Decision rules and the matched decision rule

For an ordinal model with $p_k = P(y > k)$ the expected grade is $s = \sum_k p_k$. Every model's outputs were decoded under the same rules (Supplementary Note 1). **R1** is the shipped rule: the number of leading thresholds with $p_k > 0.5$. **R2** places four cut-points on s to maximise QWK on development out-of-fold predictions, and **R3** moves R2's grade-1/2 cut to give 90% development referable sensitivity. **R4** and **R4m** fit the cut-points on the external labels, maximising QWK or macro-averaged recall, by two-fold cross-fitting; they are diagnostics of what local labels could recover, never performance. **R5** adapts the class posteriors (derived from the ordinal outputs) to the external population without labels by EM estimation of the target prior[32], and **R5b** by black-box shift estimation[33]; both decode by argmax.

The *matched decision rule* is the comparison rule used throughout: no difference between two models is attributed to the representation unless it survives refitting both models' cut-points by the same cross-fitted procedure (on development data, fold f's cut-points come from the other four folds; externally, R4). Because a per-grade verdict can depend on how the cut-

points are cross-fitted (Supplementary Note 3), every matched claim names its procedure.

2.5. Decomposing external performance

Discrimination was measured at every ordinal threshold (area under the ROC curve of p_k for $y > k$, $k = 0, \ldots, 3$) and overall by Harrell's C of s. The mapping from score to grade was described by the mean expected grade given the true grade, $\mathrm{E}[s \mid y = k]$; under a pure prior (label) shift this quantity is the same in every population, so a grade-by-grade departure from its development value indicates a conditional shift (outcome A1). To test whether image quality alone can produce the under-grading seen on DDR (A2), the development pool was re-scored out of fold after five fixed degradations of the cached image (JPEG quality 30 at 512 pixels; downsampling to 256 pixels; Gaussian blur, $\sigma = 2$ pixels; gamma 1.8 darkening; contrast reduced to 60%), and per-corpus image statistics were computed (A3).

2.6. Threshold transfer and local recalibration

A referral threshold on $P(y \geq 2)$ was fitted on development predictions for target sensitivities of 80%, 85% (the superiority endpoint of the first autonomous pivotal trial[4]), 87.2% and 95.5%, and applied unchanged externally. To size local recalibration with the operational objective, n external images (n = 25–800, 500 draws each) were drawn at random; the referral threshold was set to the largest value giving at least 85% sensitivity on those n, and sensitivity and specificity were measured on the remaining images. A draw counts as *harm* when held-out sensitivity falls below 80%. Positive and

negative predictive values and the number of referable patients missed per 10 000 screened were derived at referable prevalences of 5, 10 and 20%.

2.7. Statistics

The primary grading metric is quadratic weighted kappa (QWK)[34]; the operationally primary pair is referable sensitivity and specificity, never quoted alone. Per-grade recall, precision and F1, Cohen's κ, macro-averaged recall (as a diagnostic, never an objective), area under the curve, Brier score and expected calibration error are reported beside them. Intervals are 95% percentile intervals from 2 000 bootstrap resamples of groups (patients where identified, otherwise images); differences between models are paired bootstraps, and an interval containing zero is reported as indistinguishable, not ranked. Model rankings were compared with Kendall's τ. A difference between configurations is treated as beyond training noise only if it exceeds $2\sqrt{2}$ standard deviations of matched development QWK across five seeds of the reference model. No correction for multiplicity is applied; per-grade claims are treated as credible only when they replicate across conditions.

2.8. Software and use of AI tools in the research process

Every number in the article is generated from archived per-image predictions into a ledger from which the manuscript is compiled; no number is typed by hand. An AI assistant (Claude, Anthropic) was used to write analysis code and to run the pre-specified analyses on Kaggle; all code, analysis plans and outputs are public, and the authors verified the analyses and every cited claim against its source.

3. Results

3.1. Cohorts and models

The ten pre-registered development models reached a matched development QWK between 0.8267 and 0.8954; the selected EfficientNet-B3, chosen on development data before any external number was examined, reached 0.8954 (Table 2). The four external populations differed in their grade distributions (Table 1): referable DR made up 40.6% of APTOS, 44.9% of DDR, **??r2.deepdrid.prevref??**% of DeepDRiD and **??r2.sustech.prevref??**% of SUSTech-SYSU.

3.2. Ranking transferred to new populations; the mapping from score to grade did not

Discrimination survived the change of population at every ordinal threshold (Fig. 2a). Across the ten models the area under the ROC curve for referable disease, $P(y > 1)$, was 0.914–0.973 on APTOS, 0.921–0.955 on DDR, **??r2.deepdrid.auc1.min??**–**??r2.deepdrid.auc1.max??** on DeepDRiD and **??r2.sustech.auc1.min??**–**??r2.sustech.auc1.max??** on SUSTech-SYSU, and no threshold on any corpus fell below 0.899; Harrell's C of the expected grade was 0.948–0.959 on APTOS and 0.889–0.923 on DDR, against 0.921 on development data for the reference model.

Where the scores fell did not transfer, and it moved in opposite directions (Fig. 2b). On APTOS the models scored images of the same true grade higher than in development for at least three of the five grades in 10 of 10 models; on DDR they scored them lower in 10 of 10 (pre-registered outcome A1 held). The shift is therefore not a change in prevalence alone, under which

the score given the true grade would not move. Its shape differed between the two populations. On APTOS it was concentrated in one grade: for the reference model, mild images had a mean expected grade of 1.90 against 0.83 in development, while no-DR images scored lower (0.19 against 0.36). On DDR every grade scored lower, from 0.72 against 0.83 for mild to 2.75 against 3.05 for proliferative DR. Over-grading on APTOS and under-grading on DDR followed: the mean predicted-minus-true grade at the shipped rule was +0.14 to +0.26 on APTOS and −0.35 to −0.17 on DDR.

A referral threshold fitted on development predictions therefore did not hold its operating point (Fig. 3). Fitted for 85% sensitivity, the reference model referred 99.33% of referable patients on APTOS, 63.38% on DDR, **??deepdrid.tr.e08.B.sens??**% on DeepDRiD and **??sustech.tr.e08.B.sens??**% on SUSTech-SYSU (pre-registered O8 held). The same behaviour appeared in models trained on **??eye.dn.trainn??** EyePACS images: their referral area under the curve stayed between **??eye.auc.min??** and **??eye.auc.max??** on the six external sets, while a threshold fitted on their held-out development fold for 85% sensitivity reached **??eye.tr85.min??**–**??eye.tr85.max??**% (outcome E2 **??eye.E2.verdict??**; Supplementary Table S6). The decomposition is not an artefact of a small development set.

3.3. Why the mapping moved

Image quality could produce the DDR pattern. Re-scored out of fold after degradation, the development pool was under-graded by the reference model: **??mech.e08.best.name??** shifted the mean predicted-minus-true grade by **??mech.e08.best.dmean??** and referable sensitivity at the shipped rule by **??mech.e08.best.dsens??** points (outcome A2 **??mech.A2.verdict??**;

Supplementary Table S7). DDR images were less sharp than development images (median Laplacian variance **??stats.ddr.sharp??** against **??stats.messidor2.sharp??** for Messidor-2) and more strongly compressed (estimated JPEG quality **??stats.ddr.jpegq??**). Image quality cannot explain the APTOS pattern, in which mild images alone moved up by about one grade while no-DR images moved down. A difference in how the mild grade was assigned by APTOS's single grader is the simpler explanation, and it changes what local recalibration does there: it fits the local grading standard (Discussion).

3.4. What the decision rule can and cannot recover

Where ranking survived, much of the external loss was a property of the rule. On APTOS every development model lost the mild grade at the shipped rule (recall 1.6–14.3%, against 26.8–62.0% in development; Fig. 4a), and so did all five seeds of the reference model (5.4–6.2%; O3 held). Cut-points refitted on half of the external labels and applied to the other half (R4m) raised mild recall to 61.6–77.8% with mild F1 of 0.501–0.544 (precision 0.406 for the selected model), above the development F1 of the same model at its shipped rule (0.395). The gain was a trade, not a free repair: 342 moderate images were relabelled mild and referable sensitivity fell from 99.3% to 75.3%, because mild DR is not referable. In the reverse direction the model trained on APTOS under-graded Messidor-2 (O6 held) and referred 43.5% of referable eyes at its shipped rule while ranking them with an area under the curve of 0.890; refitted cut-points raised sensitivity to 77.9%.

Where ranking or the grade itself did not survive, the rule could not help. On DDR the mild grade was not collapsed (recall 20.5–45.2%) and no rule raised its F1 above 0.235. In the reverse direction mild recall at

the shipped rule was 34.4%, so the pre-registered predictions that a fine-tuned model would reproduce the published mild-grade collapse (O4) and that refitted cut-points would triple it (O5) were both falsified. RETFound ranked APTOS images as well as the convolutional models (area under the curve 0.947) and most of its QWK deficit there was the rule (0.703 at its shipped rule, 0.834 refitted), but on DDR its ranking itself degraded (0.855; QWK 0.646 even refitted).

Rules that need no external labels did not close the gap. Cut-points fitted on development data (R2, R3) left APTOS mild recall at 5.7–22.7% and lowered external QWK for 9 of 10 models. Label-free prior adaptation (R5) closed at least a quarter of the DDR referral-sensitivity gap between R1 and R4 in only 2 of 10 models (outcome B was falsified), as expected when the shift is in the score given the grade rather than in prevalence; decoding the adapted posteriors by argmax also never produced a mild prediction.

3.5. Model comparisons depend on the decision rule

The pre-registered prediction that the choice among deployable rules (R1–R3) would move mild recall and referable sensitivity more than the choice among models (O7) was falsified on both APTOS and DDR: models trained on the same data differed materially at their shipped rules (median within-model range 5.7 against a between-model range of 12.7 points of mild recall on APTOS, 7.1 against 11.4 points of referable sensitivity on DDR). Which model looked best, however, depended on the rule and on the metric. The ranking of models by external QWK at the shipped rule and by referable sensitivity at a fixed 90% specificity was nearly unrelated (Kendall τ -0.16 on APTOS, $+0.22$ on DDR).

Under the matched decision rule several published-style conclusions changed (Table 3). On APTOS the selected EfficientNet-B3 led the DenseNet121 reference by +0.0059 QWK [+0.0007, +0.0110] at the shipped rule and by +0.0032 [−0.0020, +0.0083] with both models refitted, and per-grade differences changed sign between operating points. On development data, linear-probe-then-fine-tune appeared to cost −5.54 accuracy points [-7.48, -3.54] at shipped cut-points and gained +1.95 [+0.18, +3.72] at matched ones. The rule is not a one-way filter: the development advantage of EfficientNet-B3 over its DenseNet121 control survived it (+0.0202 shipped, +0.0207 [+0.0105, +0.0320] matched).

The same held for an independently trained, published model family. Applying our rules to the released image-level probabilities of ORDER-DR[11] reproduced its reported Messidor-2 result exactly (QWK 0.6423 ± 0.0364; Supplementary Table S5); with external cut-points all four released models converged to QWK 0.672–0.683, so the reported advantage of the fused model over its best branch was a decision-rule effect, while mild F1 stayed at or below 0.285.

Training noise set the scale for all of these effects (O1). Matched development QWK varied with a standard deviation of 0.0043 across five seeds of one configuration (0.8646–0.8764). Between models that differed only in their seed, 15 of 50 per-grade recall comparisons on APTOS had a bootstrap interval excluding zero at the shipped rule and 29 with refitted cut-points (O2 held). An image-level bootstrap captures the sampling of test images, not the variance of training, so per-grade "effects" of the size usually attributed to architecture arise between identical architectures.

3.6. Agreement statistics cannot see the referral operating point

Holding the other cut-points at their QWK-optimal values and sweeping the referral cut (Fig. 5), every operating point of the selected model within 0.01 of its maximum QWK on DDR spanned referable sensitivity from 71.3% to 86.3%: between 773 and 1,615 of 5,626 referable patients missed, with QWK unable to tell the operating points apart. In a screening programme with 10% referable prevalence, the selected model's shipped rule on DDR would miss 297 referable patients per 10 000 screened (negative predictive value 96.8%), and cut-points refitted on local labels 188, at the cost of 521 rather than 117 false referrals (Supplementary Table S8).

3.7. How many local labels a referral threshold needs

With the operational objective — a referral threshold set on n locally labelled images for 85% sensitivity — held-out sensitivity of the selected model averaged close to target at every size, but its spread was wide at small n (Fig. 6). With 100 images 20.8% of draws on DDR and 22.2% on APTOS ended below 80% sensitivity; with 200 images the probability of harm was 9.4–15.8% across corpora and models (outcome C held), and with 400 images 5.0% on DDR. A recommendation judged by mean sensitivity alone would have endorsed sample sizes at which about one recalibration in five fails its target.

3.8. Interventions on the representation

Of 9 pre-registered interventions on the model, 8 failed their pre-specified criterion under the matched decision rule (Supplementary Fig. S1 and Table S4). Only the EfficientNet-B3 backbone improved development QWK;

the retinal foundation model, fine-tuned at its native 224 pixels, fell below a DenseNet121 at the same resolution by −0.0414 [−0.0583, −0.0238] over 5 folds (O9 held). No intervention moved the macular-oedema head, whose middle grade is supervised by 51 images (Supplementary Note 2).

4. Discussion

In the DR-grading settings studied here, an external failure separated into two components that call for different remedies. The ranking of images by severity transferred to four new populations and to models trained on twelve times more data; the mapping from score to grade did not, and its error had opposite signs on different populations because the score given the true grade moved. Where ranking survived, part of the loss was a property of the decision rule and could be recovered without retraining — the mild grade on APTOS, referral sensitivity in the reverse direction — but only as a trade between grades, and only with local labels. Where ranking degraded, as for the foundation model on DDR, no rule helped. Two of our own predictions bound the argument: deployable rules did not outweigh the choice of model, and the mild-grade collapse that the literature reads as representational blindness was specific to one corpus. A reader who sees an external failure should therefore ask, in order, whether the model still ranks the new population, where its cut-points were set and on which population, and only then what it has learned.

These results refine rather than contradict recent work. ORDER-DR separated threshold-dependent grade assignment from threshold-independent risk ranking and noted threshold shift on external data[11]; reanalysing its

released predictions shows that its reported advantage over its own branches does not survive the matched decision rule. Tabibu and Abhyankar found that the probability calibration of a binary referral model degrades across cameras mostly through prevalence, and that 100–200 labelled images restore it[12]. Our sample-size estimate agrees, for an ordinal model and a referral threshold rather than a calibration map, but the mechanism we found differs: the score given the true grade moved, so prior adaptation without labels failed, and temperature-style rescaling cannot be expected to help either, as reported for frozen encoders[8]. Per-threshold calibration has also been reported to degrade unevenly under domain shift[13], consistent with the grade-specific movement we observed on APTOS.

The central objection to this kind of analysis is that refitting cut-points on local labels may fit local label noise rather than fix the model. The two populations answer it differently. On DDR every grade moved down by a similar amount and degrading development images reproduced the direction (A2), so recalibration there corrects a change in the images. On APTOS one grade moved by about a whole step while its neighbours did not, which image quality does not produce and a difference in how a single grader assigned the mild grade would; recalibration there aligns the model with the local grading standard. Whether that is a fix depends on whether the local standard is the one the screening programme acts on — which, for a programme deploying a model, it usually is. Either way, the rule must be fitted where it will be used, on a sample large enough for its risk to be known.

The practical recommendations cost no additional training. External evaluations should report discrimination at each ordinal threshold beside

grade-level agreement, the full referral operating curve, and the transfer gap of any development-fitted threshold, since the same threshold landed +18.1 and −29.0 points from an 80% target on two populations. Model comparisons should be made under a matched decision rule that names its fitting procedure, with differences judged against the variation between seeds of one configuration. Deployment should include setting the referral threshold on a local sample: with about 200 labelled images the probability of missing an 80% sensitivity floor was below one in five on every population, and with 400 it was a few per cent, in line with advice on updating prediction models for new settings[9, 10, 35].

The study has limitations. All data are public and retrospective, the populations are defined by dataset rather than by a prospective screening programme, and APTOS and EyePACS labels come from single graders. Patient identifiers are unavailable for most corpora, so resampling is by image and fellow-eye pairs may be correlated. The external-label rules (R4, R4m) are diagnostics of what local labels could recover, not external validation; the deployable recommendation rests on the separate sample-size analysis. Referable disease was defined by DR grade alone because macular oedema is not graded in the external sets, whereas real screening refers for both; the macular-oedema head could not be validated externally and is reported only in the Supplement. The reverse direction, the seeds and the large-data models each use one or two architectures. Finally, the analysis plans were committed to version control rather than to a public registry, and the pre-specification of development runs trained before the repository history begins cannot be verified. The decision-rule effects should be confirmed prospectively, with

cut-points fitted on an initial local sample and then frozen.

5. Conclusions

External failures and apparent differences between DR grading models are routinely attributed to learned representations. In the settings studied here a substantial part of them lay in the decision rule that converts scores into grades and could be diagnosed and repaired without retraining, while the remainder reflected real losses of ranking that no rule recovers. Reporting per-threshold discrimination, full operating curves, the transfer gap of thresholds and matched-decision-rule comparisons, and setting referral thresholds on a local sample of known size before deployment, would separate the two.

CRediT authorship contribution statement

Ahmadreza Nourozi: Conceptualization, Methodology, Software, Formal analysis, Investigation, Data curation, Visualization, Writing – original draft, Writing – review & editing, Project administration. **Alireza Chegeni:** Conceptualization, Methodology, Investigation, Validation, Writing – review & editing.

Declaration of competing interest

The authors declare that they have no known competing financial interests or personal relationships that could have appeared to influence the work reported in this paper.

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Ethics statement

The study used only publicly available, de-identified fundus photographs released for research (IDRiD, Messidor-2, APTOS 2019, EyePACS 2015, DDR, DeepDRiD and SUSTech-SYSU). No new data were collected from human participants, and ethical approval was not required.

Data and code availability

All images are from public datasets. Code, the frozen splits, per-image out-of-fold and external predictions for every model, the analysis plans with their commit history, and every generated table and figure are available at `https://github.com/Ahmadrezanourozii/Automatic-detection-of-diabetic-retinopathy-and-grading-of-diabetic-macular-edema-using-CNN`.

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Declaration of generative AI and AI-assisted technologies in the manuscript preparation process

During the preparation of this work the authors used Claude (Anthropic) to draft and edit the manuscript text and to generate its figures and tables from the archived analysis outputs. After using this tool, the authors reviewed and edited the content as needed and take full responsibility for the content of the published article.

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Table 1: **Cohorts.** DR is graded on the international 5-grade scale in every corpus. No external image was used for training, model selection or threshold fitting.

Role	Corpus	Origin	Images	Grading
Development	IDRiD	India	516	3 graders (published grading)
	Messidor-2	France	1,744	adjudicated by 3 specialists
Pretraining; large-data models	EyePACS 2015	USA	35,126	single grader
External	APTOS 2019	India	3,662	single grader
	DDR	China, 147 hospitals	12,522	7 graders
	DeepDRiD (regular)	China, Shanghai	2,000 (500 patients)	challenge release
	SUSTech-SYSU	China, Shenzhen	1,151	international scale, released
Reverse direction	APTOS → Messidor-2, IDRiD		3,662 → 2,260	as above

Table 2: **Models.** Each development model changes one factor relative to the reference. Development QWK: out-of-fold DR QWK under the matched decision rule (fold-wise cross-fitted cut-points). Repository codes are given for traceability.

Model (code)	Backbone	Pixels	Factor changed	Dev. QWK
DenseNet121-448 (reference) (E08)	DenseNet121	448	reference	0.8646
DenseNet121-224 (E09)	DenseNet121	224	resolution	0.8389
DenseNet121-560 (E10)	DenseNet121	640 (560 effective)	resolution	0.8749
DenseNet121-640 native (E17NAT)	DenseNet121	640	native-resolution source images	0.8662
EfficientNet-B3 (selected) (E11FULL)	EfficientNet-B3	448	backbone (selected model)	0.8954
DenseNet121 + macula crop (E14MAC)	DenseNet121	448	macula-centred crop for the DME head	0.8595
DenseNet121 LP-FT (E15LPFT)	DenseNet121	448	linear probe, then fine-tune	0.8655
DenseNet121 CORAL head (E20CORAL)	DenseNet121	448	CORAL ordinal head	0.8569
DenseNet121 CORN head (E22CORN)	DenseNet121	448	CORN ordinal head	0.8564
ConvNeXt-T (2 folds) (E21CNXA)	ConvNeXt-tiny	448	backbone (2 of 5 folds)	0.8267
RETFound ViT-L/16 (I24FULL)	RETFound ViT-L/16	224	foundation encoder (not in the O7 set)	0.7976
DenseNet121, EyePACS-trained (EYEDN)	DenseNet121	448	trained on EyePACS	–
EfficientNet-B3, EyePACS-trained (EYEEB3)	EfficientNet-B3	448	trained on EyePACS	–

Table 3: **The matched decision rule changes model comparisons.** Differences (first minus second) with 95% paired-bootstrap intervals at the shipped rule and with both models' cut-points refitted by the same procedure (external rows: R4, two-fold cross-fitted on APTOS labels; development rows: fold-wise cross-fitted). *Interval excludes zero. A per-grade verdict that depends on the cross-fitting procedure is reported in Supplementary Note 3.

Comparison	Shipped rule	Matched decision rule	Verdict
EfficientNet-B3 vs DenseNet121, APTOS QWK	+0.0059 [+0.0007, +0.0110]	+0.0032 [−0.0020, +0.0083]	advantage dissolves
recall, moderate (points)	+1.4	+4.0*	significance changes
recall, severe (points)	−1.0	−10.3*	significance changes
recall, proliferative (points)	+8.1*	+6.8*	persists
LP-FT vs fine-tuning, development accuracy (points)	−5.54 [−7.48, −3.54]	+1.95 [+0.18, +3.72]	reverses
EfficientNet-B3 vs DenseNet121, development QWK	+0.0202 [+0.0071, +0.0329]	+0.0207 [+0.0105, +0.0320]	survives

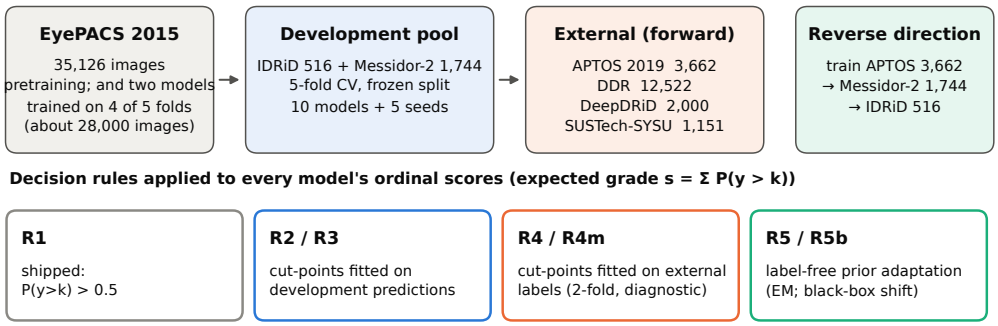


Figure 1: **Study design.** Development models were trained on a pooled IDRiD and Messidor-2 set under a frozen 5-fold split (after pretraining on EyePACS) and evaluated without further tuning on four external populations; a model trained on APTOS was evaluated in the reverse direction, and two models were trained on about 28 000 EyePACS images. Every model's ordinal outputs were decoded under the same decision rules: R1, the shipped default; R2 and R3, fitted on development predictions; R4 and R4m, fitted on external labels by two-fold cross-fitting (diagnostic only); and R5 and R5b, label-free adaptation to the external population.

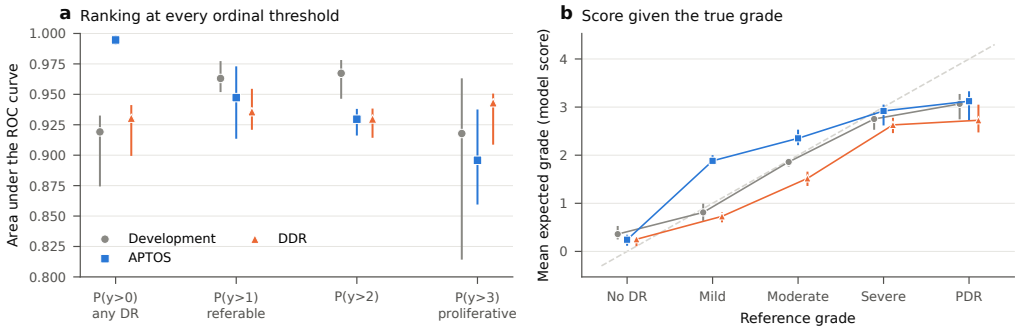


Figure 2: **Ranking transferred; the score given the true grade did not. a**, Area under the ROC curve of each ordinal output $P(y > k)$ in development (out of fold) and in each external population: median (marker) and range (line) across the ten pre-registered models. **b**, Mean expected grade $\mathrm{E}[s \mid y]$ for images of each reference grade; under a pure change in prevalence these curves would coincide. APTOS images of the mild grade score about one grade higher than in development, whereas DDR images of every grade score lower. Colours are paired with marker shapes.

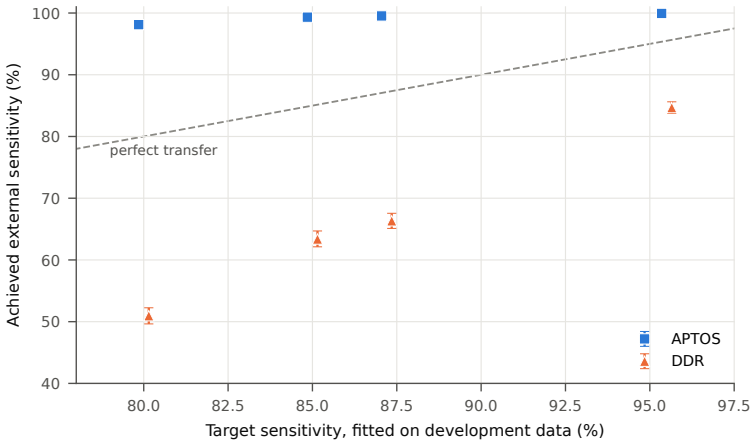


Figure 3: **Referral thresholds fitted on development data do not hold their operating point.** A threshold on $P(\text{grade} \geq 2)$ was fitted on the reference model's development out-of-fold predictions for each target sensitivity (x-axis) and applied unchanged to each external population; points show the achieved sensitivity with 95% bootstrap intervals, and the diagonal is perfect transfer. Targets: 80%; 85% (the superiority endpoint of the first autonomous pivotal trial); 87.2% (that trial's achieved sensitivity); 95.5%. The same thresholds over-refer where the models over-grade (APTOS) and under-refer where they under-grade (DDR).

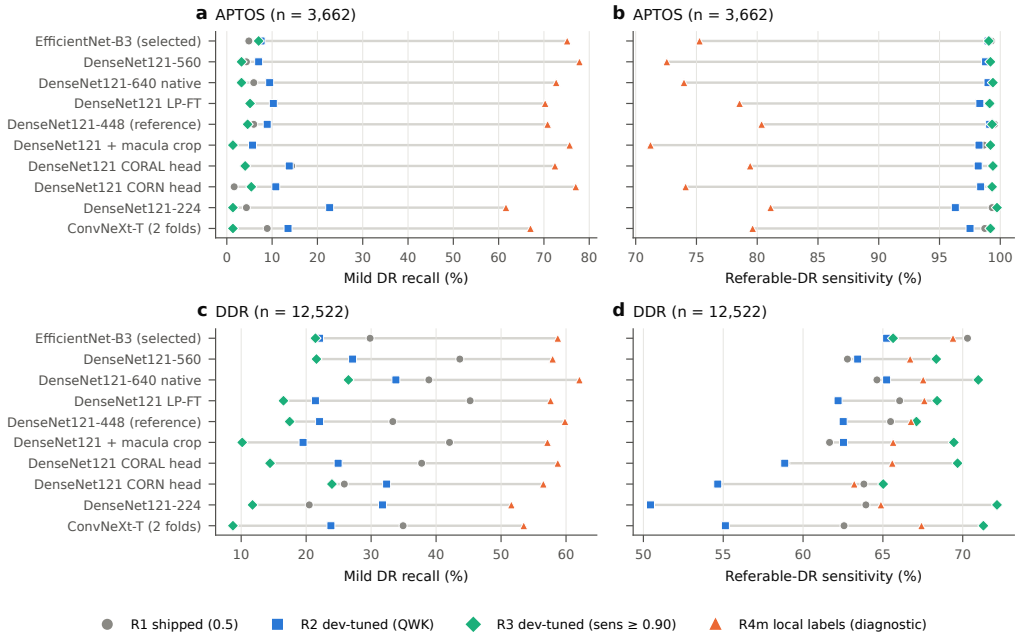


Figure 4: **External mild-grade recall and referral sensitivity of every pre-registered model under every decision rule.** Rows are models (Table 2) ordered by development QWK; markers show the same model under four rules. **a**,**b**, APTOS 2019; **c**,**d**, DDR. On APTOS, mild recall at the shipped rule (R1) is low in every model and is restored by cut-points refitted on external labels (R4m, diagnostic) at the cost of referral sensitivity, because mild DR is not referable; rules fitted on development data (R2, R3) do not restore it. On DDR the mild grade is not collapsed and no rule recovers much. Models differ at a fixed rule by more than the deployable rules differ within a model (O7 falsified).

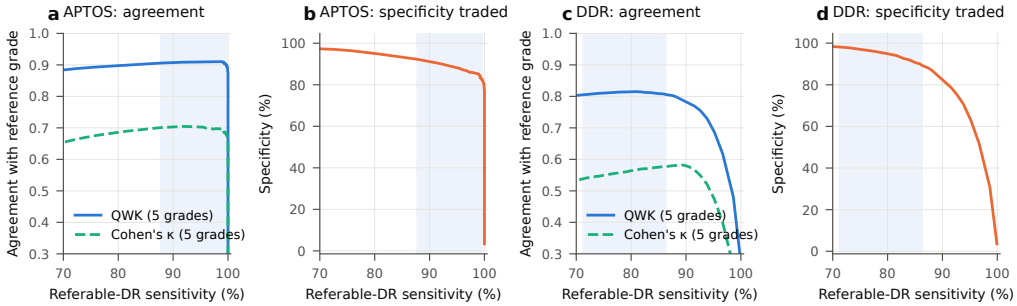


Figure 5: **Agreement statistics cannot see the referral operating point.** **a**,**c**, QWK and Cohen's κ of the selected model as the referral cut-point is swept with the other cut-points at their QWK-optimal values, against the referable sensitivity it produces, on APTOS (**a**) and DDR (**c**). The shaded band contains every operating point whose QWK is within 0.01 of the maximum. **b**,**d**, the specificity that the same sweep trades for sensitivity.

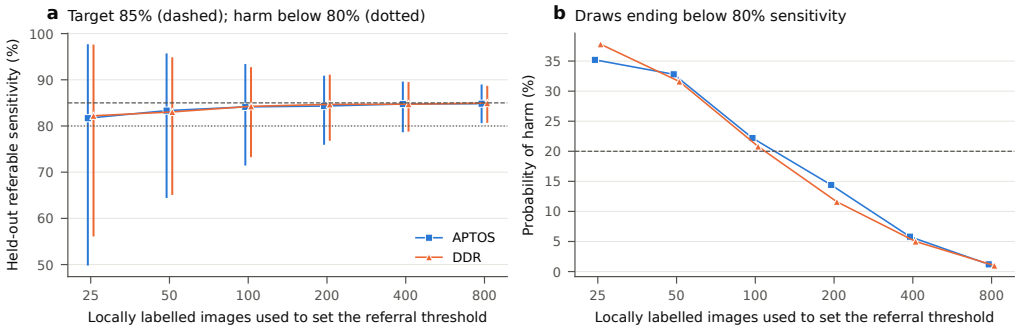


Figure 6: **How many locally labelled images a referral threshold needs.** For the selected model, the referral threshold was set on n randomly drawn labelled images of each external population to give at least 85% sensitivity on those images and evaluated on the remaining images (500 draws per size). **a**, mean held-out sensitivity with the 2.5th–97.5th percentile range of draws. **b**, probability that a draw ends below 80% sensitivity (harm).